PORTFOLIO PROJECT · HEALTH MANAGEMENT

Clinical Documentation and AI Scribes in Health Management

What the 2026 evidence says about time, burnout, accuracy, cost and consent

Neha · Compiled October 25, 2026

Ambient AI scribes lower clinician burnout and trim documentation time modestly, but managers must govern accuracy, billing and patient consent to keep the gains.

Contents

Section	PDF page
Executive summary	2
Full report: Clinical Documentation and AI Scribes in Health Management	3
Data analysis: What the Numbers Say About AI Scribes	10
Slide deck (12 slides)	13
Resume bullets and online links	25

Online versions

- Interactive evidence explorer
- Slide deck (online)
- Full report (online doc)

These online pages are private to the account that published them, so they open only for the owner unless shared. The interactive explorer lets readers switch between six topics (time, burnout, accuracy, money, consent, nursing) and cannot be shown in a PDF.

About the evidence

Figures come from search results and fetched page summaries retrieved October 16 to 20, 2026, and several sources are secondary summaries. Verify a number at its original study before citing it. The data analysis uses published summary figures because row-level datasets could not be downloaded in this workspace.

Executive summary · Portfolio project · Health management

Clinical Documentation and AI Scribes

What the 2026 evidence says about time, burnout, accuracy, cost and consent

Ambient AI scribes lower clinician burnout and trim documentation time modestly, but managers must govern accuracy, billing and patient consent to keep the gains.

Documentation is the administrative burden clinicians most want AI to remove, and ambient scribes are the most mature tool. The evidence below is from the full report, October 2026.

KEY FINDINGS

- **Adoption is broad but shallow.** 68% of surveyed health systems have adopted AI note-taking, but only 32% of users in a five-center study used a scribe in more than half their visits.
- **Time savings are real but modest.** The largest study (observational, about 1,800 adopters) found 16 fewer documentation minutes per 8 hours of scheduled care, about 1–2 minutes per visit. Heavy users saved 2–3 times more. After-hours record time did not change significantly.
- **Burnout relief is the stronger benefit.** Pilot users reported 21% lower burnout at Mass General Brigham and about 31% lower at Emory. A randomized trial found less work exhaustion but no significant gain in professional fulfillment.
- **Most notes are sound; omissions lead errors.** In a 31-physician pilot, 94.7% of 356 evaluated notes had no significant errors. Omissions affected 18% of notes, hallucinations 11.5%, and 5.3% carried serious or imminent risk.
- **Revenue gains invite payer scrutiny.** One study estimates at least $167 more visit revenue per clinician per month. Trilliant found top-level new-patient codes up 12–20 points, with no control group. Cigna began downcoding some visits in October 2025.
- **Consent rules vary and are being tested.** A proposed class action (filed April 8, 2026) alleges recording without clear consent at Sutter Health and MemorialCare; the claims are unproven. Rhode Island has the first scribe-specific state law.
- **Nursing is next, with thin evidence.** Mount Sinai Medical Center extended ambient charting to bedside nurses in July 2026, but a randomized nursing trial only began recruiting in May 2026.

RECOMMENDATIONS FOR HEALTH MANAGERS

1. **Pilot with controls.** Compare users with non-users using EHR audit logs.
2. **License for active use.** Negotiate on use, since heavy users save the most.
3. **Audit every note.** Treat clinician review as a safety control and sample for omissions.
4. **One consent rule.** System-wide script and opt-out, checked against each state.
5. **Govern coding.** Baseline the code mix and audit before booking revenue.
6. **Protect the time.** Track burnout and access; do not refill saved minutes with heavier schedules.

Ownership matters as much as the list: only 18% of health systems report mature AI governance, so assign one accountable leader for documentation AI.

LIMITS OF THE EVIDENCE

Most outcomes are self-reported, many studies draw on early adopters, and the largest time study is observational. Figures come from search results and fetched page summaries retrieved October 16, 2026, and several sources are secondary summaries. Verify a number at its original study before citing it. The full report lists all sources with links.

Clinical Documentation and AI Scribes in Health Management

Oct 16, 2026 · @Neha

Ambient AI scribes lower clinician burnout and trim documentation time modestly, but managers must govern accuracy, billing and patient consent to keep the gains.

Why documentation is a management problem

Documentation is the administrative burden clinicians most want AI to remove, and the one where AI tools are most mature.

Clinicians spend much of each visit, and many evenings, working in the electronic health record. Experts have long linked that work to bureaucracy-driven burnout, and a physician quoted by Healio says most of a clinical encounter is spent typing (Healio). In the AMA's 2026 survey, 70% of physicians saw AI as a way to automate tasks that feed burnout (Healthcare Leaders Magazine).

For managers the question goes beyond comfort. Health systems say they are adopting AI to address workforce constraints and financial pressure together (Fierce Healthcare), and documentation time touches both. A summary of a multicenter quality improvement study reports self-reported burnout falling from 51.9% to 38.8% after 30 days of scribe use (MemX). The sections below ask whether that promise holds up, what it costs, and what could go wrong.

Adoption: broad, but shallow in daily use

Scribes have become the lead AI use case in health systems, yet many clinicians use them in only a minority of visits.

- **Systems.** Clinical note-taking reached 68% adoption among surveyed health systems, with 62% growth in a year (AI2 Work).
- **Physicians.** The share of physicians regularly using voice-based documentation tools, including scribes, rose from 20% in April 2025 to 29% in January 2026, per a Doximity survey of 3,151 physicians (MemX).
- **Scale.** Kaiser Permanente's Northern California group enabled the technology for 10,000 physicians in October 2023 (Epocrates). Epic launched its own AI charting in February 2026, alongside products from Abridge, Microsoft, Ambience and Freed (BERI).
- **Depth of use.** Only 32% of users in a five-center study used the tool in more than half of their visits, and a randomized trial saw scribes used in 29.5% to 33.5% of visits (same BERI source).

The last point matters for every figure that follows: results depend heavily on how often clinicians actually use the tool.

Time savings: real but modest

Measured time savings are roughly 1 to 2 minutes per visit, well below what vendor demonstrations suggest.

Study	Design and sample	Time result
JAMA five-center study (HealthExec)	Observational; Mass General Brigham, Emory, UCSF, UC Davis and Yale New Haven; about 1,800 adopters among 8,500+ clinicians	16 fewer documentation minutes and 13 fewer record minutes per 8 hours of scheduled care; heavy users (50%+ of visits) saw roughly two to three times the savings
Same study, per STAT	As above	No significant change in record time outside working hours
NEJM AI randomized trial (BERI)	238 outpatient physicians, three arms	Nabla cut time in notes by 9.5%; DAX showed no statistically significant effect
Chicago Abridge cohort (BERI)	One institution, matched controls	1.8 minutes saved per appointment (15.9%)
Permanente deployment (BERI)	7,260 physicians over 63 weeks	15,791 hours saved, about 2.2 hours per physician by BERI's own arithmetic

The JAMA authors also associate adoption with a 1.7% rise in weekly visit volume (HealthExec), and STAT notes primary care and female clinicians benefited more.

Two cautions follow. The savings are concentrated among frequent users, and the strongest large-scale evidence is observational. One analysis concludes that the data do not support the hour-a-day savings that vendor demonstrations imply (BERI).

Burnout and well-being: the stronger benefit

The burnout evidence is more consistent than the time evidence, which suggests relief comes from less mental load, not only fewer minutes.

- **Pilot surveys.** Burnout fell 21% at Mass General Brigham and about 31% at Emory among pilot users, and a study author said she knows of no other intervention that moves burnout this much (HealthDay). A multicenter study of 451 physicians linked scribes to lower burnout, cognitive load and documentation time (Healio).
- **Randomized evidence.** A stepped randomized trial found a significant drop in work exhaustion without hurting documentation quality, but no significant gain in professional fulfillment (2 Minute Medicine).

- **Product comparison.** A 2026 crossover trial of two scribe products found both reduced burnout, with no meaningful difference between them, though one product scored higher on satisfaction and time savings (JAMIA, via Duke Scholars).

STAT notes that studies finding large well-being gains despite small time savings imply that minutes saved may not be the main benefit (STAT). The limits are that most results are self-reported and many come from early adopters who chose to try the tool. Scribes also leave other burdens untouched: a physician quoted by Healio points to patient portal messages as a major cause of burnout that scribes do not stop.

Accuracy and patient safety

Most AI-drafted notes are sound, but omissions are the most common error, and a small share of errors carry serious risk.

A two-month pilot with 31 physicians produced 7,545 notes, of which physicians evaluated 356. Of those, 337 (94.7%) had no significant errors (eScholarship). Errors that did appear:

Error type	Notes affected (of 356)	Share
Accidental omission	64	18%
Hallucination	41	11.5%
Accidental inclusion	33	9.3%
Bias	4	1.1%

Physicians rated 19 notes (5.3%) as containing errors with serious or imminent risk to patients (same pilot). Physicians changed a median 9% of AI-generated words, and about 15% of notes went unedited, which shows how much review depends on the individual clinician.

Other sources point the same way. A simulated study cited by BERI found error rates of 16.7% to 23.3% for four commercial platforms and 46.1% for a free one, with about three quarters of errors being omissions (BERI). In Kaiser Permanente's rollout, a scribe once recorded a planned prostate exam as already done and noted a hand, foot and mouth diagnosis the physician had not made (Epocrates). The management implication is that clinician review is a safety control, not a courtesy, and it needs auditing.

Money: cost, revenue and the coding arms race

Scribes may raise revenue through fuller notes and higher visit codes, but that same effect is drawing payer scrutiny.

- **Cost.** Published prices are scarce. Freed lists $39, $79 and $119 per clinician per month, aimed at small non-Epic practices; most other vendors, including Epic, do not publish prices. BERI advises negotiating on active use, since licensing every clinician pays for tools many will rarely use (BERI).

- **Revenue.** The JAMA five-center authors estimate at least $167 more evaluation and management revenue per clinician per month. None of the sites required adopters to book more patients, so the gain may come from unused template time or higher-level coding (HealthExec). A UCSF study of 1.2 million visits found users billed 1.81 more relative value units a week and saw about 0.80 more patients, with no rise in denied claims (DH Insights).
- **Coding intensity.** A Trilliant Health analysis of six systems found new-patient visits billed at the top two levels rose 12 to 20 percentage points after adoption. It had no control group and cannot separate accurate documentation from overstated documentation (HLTH).
- **Payer response.** Nearly 70% of health plan actuaries surveyed by PwC ranked AI documentation and coding tools among their top three cost drivers. Cigna began automatically lowering many mid- and high-level visit claims by one level in October 2025 unless documentation clearly supported them (DH Insights). Blue Cross Blue Shield research found postpartum anemia coding tripled at fast-growing hospitals while transfusion rates stayed flat, though that report comes from an insurer (HLTH).

Overbilling is not established. The UCSF authors cannot tell whether extra units reflect more services, more accurate coding or overstatement, and AI is not the largest driver of rising premiums. Still, a revenue case built on higher coding is exposed to audits and downcoding, so managers should plan for compliance review before counting that revenue.

Consent, privacy and the law

Recording exam-room conversations creates consent and privacy exposure that varies by state and is now being tested in court.

- **Litigation.** A proposed class action filed April 8, 2026 in federal court in Northern California alleges Sutter Health and MemorialCare used Abridge's scribe to record visits without clear patient consent. It cites California's medical privacy and invasion of privacy laws, which require all-party consent for recording confidential communications, and the federal Wiretap Act. Sutter says it is reviewing the case and takes privacy seriously. These are unproven allegations (TechTarget).
- **State law.** Rhode Island is described as the first state with a law aimed at ambient scribes, requiring disclosure to patients, a way to opt out and clinician review of AI-generated documentation. Texas and Utah rely on broader AI disclosure laws, and most other states do not address scribes directly. Recording consent rules differ by state, and HIPAA applies everywhere (JUCM).
- **Practice.** Mount Sinai Medical Center in Miami Beach tells patients when its nursing scribe is in use, lets them decline, and keeps anything out of the record until a nurse or provider approves it (AI2 Work).

Because rules vary, a single system-wide consent script and opt-out process, checked against each state where the organization operates, is safer than leaving consent to individual clinicians.

Beyond physicians: nursing documentation

Ambient documentation is reaching bedside nurses, but nursing-specific evidence is still thin.

On July 28, 2026, Mount Sinai Medical Center in Miami Beach announced that inpatient nurses can use Epic's ambient tool to turn bedside conversation into care-plan documentation. It is reported to be the fourth U.S. health system to extend the tool to nursing, after Houston Methodist in March 2026, and it works in English and Spanish. Leaders approved the rollout after physicians reported nearly 30% less chart time per encounter (AI2 Work).

The same source lists the caveats:

- Nursing outcome data is mostly vendor-reported or internal, and a randomized trial at the University of Wisconsin-Madison only began recruiting in May 2026.
- Nursing documentation consists of many structured flowsheet fields per shift, which is harder for AI than a physician note.
- Saved time can be absorbed by heavier patient loads unless leadership protects it.
- Accuracy is the top barrier to AI use among nurses, cited by 33% in a 2026 McKinsey survey.

This is an editorial summary of other sources, so treat the figures as indicative.

Context: the other management challenges

In this project's judgment, documentation has better evidence than the other four areas where health systems are applying AI, which are summarized here only for context.

Challenge	One-line status	Source
Prior authorization and denials	Physicians average 39 prior authorization requests a week; reported AI gains are mostly vendor figures	ADS
Capacity and patient flow	UCHealth reports 1,700+ more patients treated with no added beds, from a hospital and vendor webinar	Becker's
Workforce and cost	Executives report 3.5x average ROI, self-reported in a survey co-authored by an investor	HLTH
Governance and safety	88% of health systems use AI, but only 18% have mature governance	Healthcare Finance News

The governance gap applies directly to scribes: a tool used by most health systems still needs an owner, an audit process and an incident route.

Recommendations for health managers

Seven actions follow from the evidence above; they are this report's synthesis rather than findings of any one study.

1. **Pilot with a control group.** Compare scribe users with similar non-users using EHR audit logs, not only surveys, as BERI recommends.
2. **License for active use.** Heavy users saw two to three times the savings, so negotiate on usage and train clinicians to reach that level.
3. **Treat note review as a safety control.** Require review before signing, audit a sample of notes for omissions and hallucinations, and track how much clinicians edit.
4. **Set one consent and opt-out standard.** Have counsel check it against each state's recording and disclosure rules, given the pending litigation and Rhode Island's scribe law.
5. **Govern coding from day one.** Record the coding mix before rollout, monitor changes, run compliance audits, and keep unreviewed coding gains out of the business case.
6. **Protect the time saved.** Measure burnout, after-hours record time and patient access, and avoid filling freed minutes with heavier schedules. Plan separately for burdens scribes miss, such as patient portal messages.
7. **Pilot nursing carefully.** Check structured flowsheet accuracy and wait for controlled results before scaling.

Ownership matters as much as the list: assign one accountable leader for documentation AI, because only 18% of health systems report mature AI governance.

Limits of this evidence

The conclusions are directional because the underlying studies have real gaps.

- Most outcomes are self-reported, and many studies draw on early adopters who chose to participate.
- The largest time-savings study is observational. BERI describes the evidence as mixed and mostly vendor-influenced or observational (BERI).
- Kaiser Permanente researchers note that studies measure time and finances well but say far less about patient experience, population outcomes and health equity (HealthExec).
- Payer responses and the consent litigation are unsettled, and several coding findings come from insurers or lack control groups.
- Several sources are secondary summaries (BERI, MemX, AI2 Work, DH Insights). Figures come from search results and fetched page summaries retrieved October 16, 2026, so verify a number at its original study before citing it.

Sources

Retrieved October 16, 2026 from search results and fetched page summaries.

- Healio on AI scribes and physicians
- Healthcare Leaders Magazine on the AMA physician AI survey

- Fierce Healthcare on Eliciting Insights 2026 survey
- MemX on scribes, burnout and the Doximity survey
- AI2 Work on health system AI adoption
- AI2 Work on Mount Sinai nursing rollout
- Epocrates on the Kaiser Permanente scribe study
- BERI on ambient scribe evidence and pricing
- HealthExec on the JAMA five-center study
- STAT on modest time savings
- HealthDay on AI scribes and burnout
- 2 Minute Medicine on the NEJM AI scribe trial
- JAMIA ambient scribe crossover trial (Duke Scholars)
- eScholarship on the quality of AI-generated clinical notes
- DH Insights on scribes, revenue and insurer response
- HLTH on scribes and coding intensity
- TechTarget on the Sutter Health and MemorialCare class action
- JUCM on state AI scribe laws
- ADS state of revenue cycle, mid-2026
- Becker's on UCHealth patient flow
- HLTH on the Bain and Bessemer ROI scorecard
- Healthcare Finance News on AI outpacing governance

Data analysis · Portfolio project · Health management

What the Numbers Say About AI Scribes

A quantitative look at burnout, note accuracy, time savings and cost, using public published figures

Bottom line: scribe benefits are real but small next to how much physician burnout swings by itself, notes need review because serious errors are not rare, and the business case holds only if a good part of the revenue gain survives payer scrutiny.

Data used. No row-level dataset could be downloaded in this workspace, so the analysis uses published summary figures: the AMA/Mayo/Stanford national physician burnout surveys (six years), one pilot study of note accuracy (356 notes), and the time-savings and price figures already in the report. Every input, source and formula is in the workbook, so each result below can be traced and changed.

1. Burnout moves far more than any one intervention

Finding. National burnout ranged from 38.2% (2020) to 62.8% (2021), a spread of 24.6 points, with a mean of 48.3% and a standard deviation of 8.8 points across the six surveys. The 2023 rate (45.2%) is only 0.3 points below 2011. About 72% of the 2020 to 2021 jump had reversed by 2023.

Scale of the scribe effect. Applying the pilot-user reductions (21% at Mass General Brigham, 31% at Emory) to the 2023 rate would imply 35.7% and 31.2%, which is 2.5 and 7.0 points below the 2020 low. That would be a large change, but pilot users chose to take part and the results were self-reported, so the real system-wide effect is likely smaller.

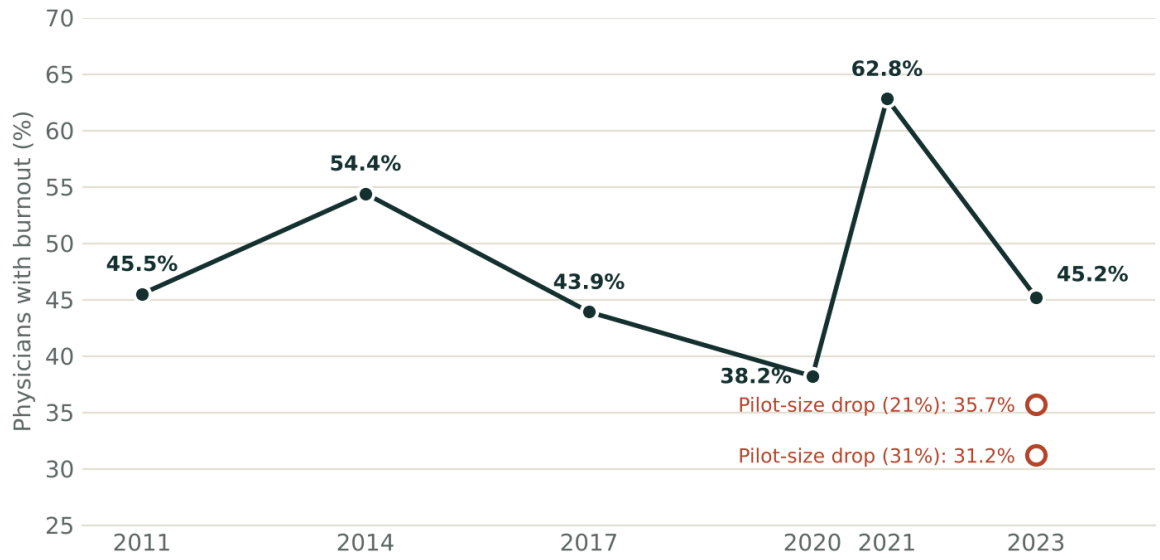


Figure 1. U.S. physician burnout by survey year (dark line) and what the 2023 rate would be under pilot-size reductions (open circles, illustrative only).

2. Notes need review: serious errors are about 1 in 19

Finding. In the 356 notes physicians evaluated, 94.7% (95% interval 91.8% to 96.6%) had no significant errors. 5.3% (3.4% to 8.2%) had an error rated serious or imminent risk. Omissions were the most common error at 18.0% (14.3% to 22.3%). Because one note can have several error types, between 18.0% and 39.9% of notes had at least one of the four types.

What it means at scale. For a clinician writing 20 notes a day over 230 days (4,600 notes a year), the pilot rate would mean about 246 notes a year with a serious-risk error (158 to 376) if nothing caught them. It is not clear

whether physicians rated notes before or after their own edits, so this is the case for review and audit, not a forecast.

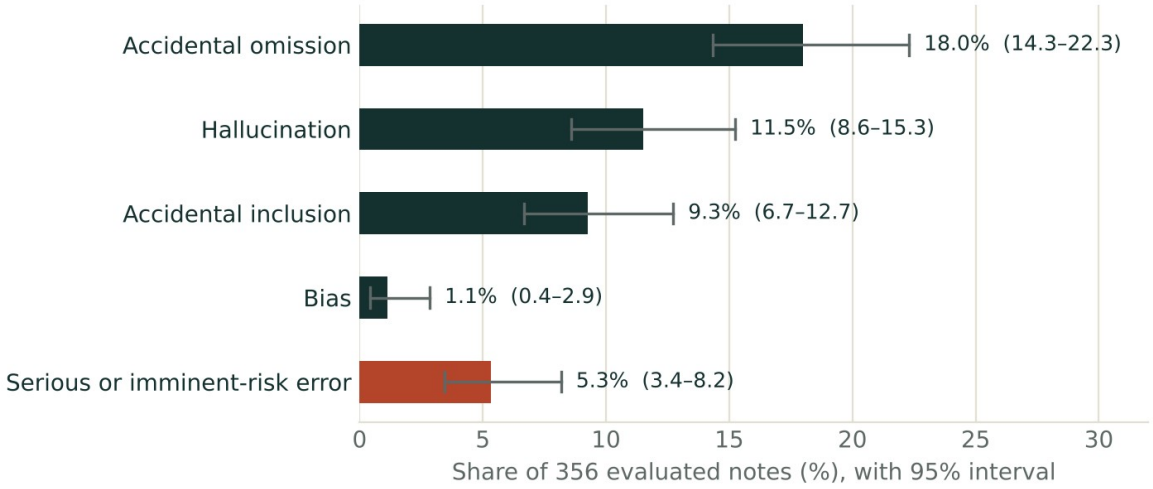


Figure 2. Share of 356 evaluated notes with each error type, with 95% Wilson intervals. Bias has only 4 notes, so its interval is wide.

3. The time-savings studies are not measuring the same thing

Finding. The JAMA five-center result of 16 minutes per 8 hours of scheduled care equals 3.3% of that time; heavy users (2 to 3 times the saving) would save about 40 minutes. The Chicago cohort saved 1.8 minutes per appointment, which implies a baseline of about 11.3 minutes. These two are broadly consistent.

A warning sign. The Permanente totals (15,791 hours across 7,260 physicians over 63 weeks) work out to 2.2 hours per physician, or only about 2.1 minutes per week. That is roughly 7.7 times lower than the JAMA figure, so it almost certainly counts something narrower and should not be compared with the others without checking the original definition.

4. Cost: the break-even depends on price and payer response

Time value. At 160 scheduled care hours a month, an average user saves about 5.3 hours and a heavy user about 13.3 hours. At the published Freed list prices that is $7.31, $14.81 and $22.31 per hour saved for an average user, and $2.92 to $8.93 for a heavy user.

Revenue. Against the lower-bound estimate of $167 extra revenue per clinician per month, a license pays for itself even if 77% of that gain disappears at $39, 53% at $79 and 29% at $119. A hypothetical $150 price tolerates only 10%, and $200 loses money even if all of the gain holds. Because part of the gain may come from higher-level coding that payers are already pushing back on, the price a health system negotiates matters more than the headline revenue number.

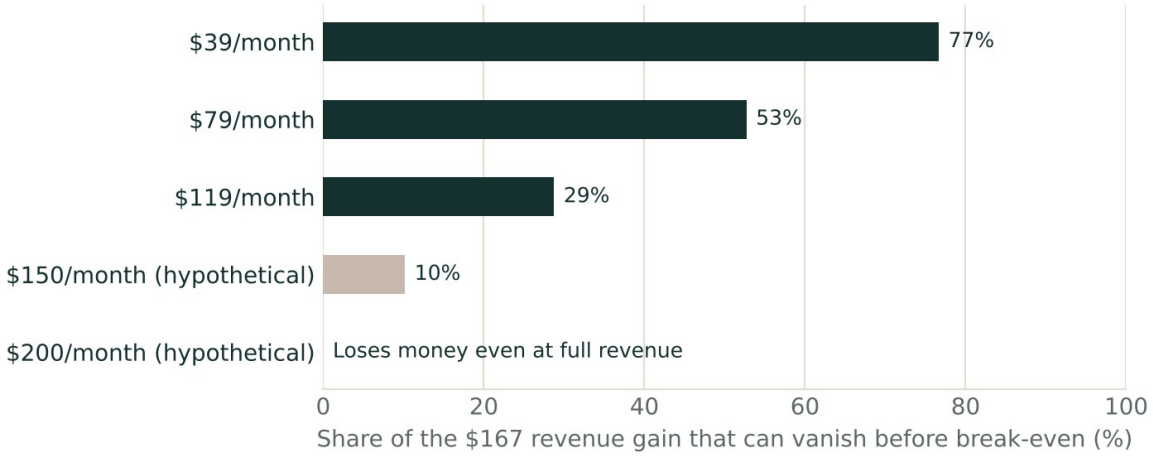


Figure 3. How much of the revenue gain can be lost before a license stops paying for itself. Prices of $39, $79 and $119 are published; $150 and $200 are hypothetical.

What this means for health managers

- **Judge scribes on well-being first.** The burnout effect is the largest benefit, but it needs controlled measurement, not pilot-user surveys.
- **Build review into the workflow.** With serious-risk errors near 5% in the pilot, clinician sign-off and audit sampling are part of the cost of the tool.
- **Negotiate on price and active use.** Break-even is comfortable at published small-practice prices and fragile at higher ones.
- **Check each study's definition of time saved.** Totals and per-visit figures can differ by a factor of several.

Limits

Six burnout surveys are too few to test trends statistically, so the burnout section is descriptive. The error rates come from one pilot of 31 physicians, and a different product could differ. Inputs were taken from search results and fetched page summaries retrieved October 16 to 20, 2026, and several come from secondary summaries; verify a number at its original study before citing it. The 20 notes a day, 230 days a year and 160 hours a month figures are assumptions, shown as blue inputs in the workbook. Nothing here shows that scribes cause the changes described.

Sources: Mayo Clinic Proceedings burnout series via HC Innovation Group and the AMA; HealthExec (JAMA five-center study); BERI (Chicago, NEJM AI and Permanente figures; Freed prices); eScholarship (note accuracy pilot); HealthDay and Healio (pilot burnout results); DH Insights (Cigna downcoding).

PORTFOLIO PROJECT · HEALTH MANAGEMENT

Clinical Documentation and AI Scribes in Health Management

What the 2026 evidence says about time, burnout, accuracy, cost and consent

October 16, 2026

ADOPTION

Widely adopted, lightly used

68%

of surveyed health systems have adopted AI note-taking, up 62% in a year

AI2 Work, on Eliciting Insights data

81%

of physicians use AI in some form, more than double the 2023 share

AMA survey, via Healthcare Leaders Magazine

32%

of scribe users used it in more than half their visits

Five-center study, via BERI

Every result that follows depends on how often clinicians actually use the tool.

TIME SAVINGS

Real, but modest

16 min

fewer documentation minutes per 8 hours of scheduled patient care

1–2 min

saved per visit, with no significant change in after-hours record time

2–3x

larger savings for clinicians who used a scribe in half or more of visits

Vendor demonstrations that imply an hour a day are not supported by the measured data.

Sources: JAMA five-center study via HealthExec; STAT; BERI.

THE EVIDENCE

What the main studies found

Study	Design	Time result
JAMA five-center	Observational, about 1,800 adopters	16 fewer documentation minutes per 8 hours
NEJM AI trial	Randomized, 238 physicians	Note time down 9.5% for one product, no significant effect for the other
Chicago Abridge cohort	One site, matched controls	1.8 minutes saved per visit
Permanente rollout	7,260 physicians, 63 weeks	About 2.2 hours saved per physician (BERI estimate)

The strongest design is randomized; the largest study is observational.

Sources: HealthExec; BERI.

BURNOUT

Burnout relief is the stronger benefit

21%

lower burnout at Mass General Brigham

Pilot-user survey

31%

lower burnout at Emory Healthcare

Pilot-user survey

Limits

- A randomized trial found less work exhaustion but no significant gain in professional fulfillment
- Most results are self-reported by early adopters
- Patient portal messages are left untouched

Sources: HealthDay on JAMA Network Open; 2 Minute Medicine on NEJM AI; Healio.

ACCURACY

Most notes are sound; omissions lead errors

94.7%

of 356 evaluated notes had no significant errors

5.3%

of notes had errors rated serious or imminent risk

ERRORS BY TYPE

Error	Share of notes
Omission	18%
Hallucination	11.5%
Inclusion	9.3%
Bias	1.1%

Source: eScholarship pilot, 31 physicians and 7,545 notes; 356 notes evaluated.

MONEY

Revenue gains invite payer scrutiny

$167

more visit revenue per clinician per month, a conservative estimate

JAMA five-center study

12–20

percentage-point rise in top-level new-patient visit codes at six systems

Trilliant Health, no control group

~70%

of plan actuaries ranked AI documentation and coding tools a top-three cost driver

PwC, via DH Insights

Overbilling is not established, but revenue built on higher coding invites audits and downcoding, as Cigna's October 2025 policy shows.

Sources: HealthExec; HLTH; DH Insights.

CONSENT AND PRIVACY

Rules vary and are being tested

Litigation

A proposed class action filed April 8, 2026 alleges recording without clear consent at Sutter Health and MemorialCare. The claims are unproven.

Rhode Island

The first scribe-specific state law requires disclosure to patients, a way to opt out and clinician review of notes.

A patchwork

Texas and Utah use broader AI disclosure laws. Recording consent rules differ by state, and HIPAA applies everywhere.

Sources: TechTarget; JUCM.

NURSING DOCUMENTATION

The next frontier, with thin evidence

4th

U.S. health system to extend ambient charting to bedside nurses, at Mount Sinai Medical Center in July 2026

33%

of nurses cite accuracy as the top barrier to AI use

McKinsey 2026 survey

RCT

A randomized nursing trial at UW–Madison only began recruiting in May 2026

Nursing notes are many structured fields per shift, which is harder for AI than a physician note.

Source: AI2 Work editorial summary; treat figures as indicative.

CONTEXT

Where documentation fits

Challenge	State of the evidence
Documentation	Several trials and large pilots; the strongest of the five
Prior authorization and denials	Widely adopted; gains mostly vendor-reported
Capacity and patient flow	Hospital and vendor case reports
Workforce and cost	Self-reported executive surveys
Governance and safety	Only 18% of health systems report mature governance

Ranking is this project's judgment. Sources: ADS; Becker's; HLTH; Healthcare Finance News.

RECOMMENDATIONS

Six actions for documentation AI

1 Pilot with controls

Compare users with non-users using EHR audit logs.

2 License for use

Heavy users save 2–3x more; negotiate on active use.

3 Audit every note

Treat review as a safety control; sample for omissions.

4 One consent rule

System-wide script and opt-out, checked by state.

5 Govern coding

Baseline the code mix; audit before booking revenue.

6 Protect the time

Track burnout and access; do not refill saved minutes.

Also: pilot nursing documentation carefully and wait for controlled results.

METHOD AND LIMITS

How to read these numbers

- Figures come from search results and fetched page summaries retrieved October 16, 2026
- Several sources are secondary summaries, so verify a number at its original study before citing it
- Most outcomes are self-reported, and the largest time study is observational
- The written report holds the full source list with links

APPENDIX

Resume bullets

AI and Clinical Documentation Burden: Health Management Portfolio Project | October 2026

- Researched how ambient AI scribes affect clinical documentation burden, synthesizing 20+ sources (peer-reviewed studies, health system data, industry surveys) into a 7-page management report with seven recommendations for health leaders.
- Analyzed evidence across time savings, burnout, accuracy, cost and consent. Found that measured savings were modest (about 1–2 minutes per visit) while burnout relief was the stronger benefit.
- Built a quantitative model in Excel with 79 formulas, including 95% confidence intervals, to test note accuracy. It found serious-risk errors in about 5% of notes (95% interval 3.4–8.2%), which supports requiring clinician review.
- Built a license-cost break-even analysis showing the tool still pays for itself at $79 a month even if about half of the estimated revenue gain is lost to payer downcoding.
- Spotted a 7.7x inconsistency between two published time-savings figures and documented it as a limit on comparing studies.
- Presented findings in a 12-slide deck with speaker notes and a one-page executive summary, and built an interactive web page that lets readers explore the evidence by topic.

Online versions

- Interactive evidence explorer
- Slide deck (online)
- Full report (online doc)

Also available as separate files: report (Word and PDF), slide deck (PowerPoint), one-page summary (Word and PDF), data analysis brief (Word and PDF) and the analysis workbook (Excel).